

# NEW YORK STATE DEPARTMENT OF HEALTH

## Communicable Disease Reporting Requirements

Reporting of suspected or confirmed communicable diseases is mandated under the New York State Sanitary Code (10NYCRR 2.10,2.14). The primary responsibility for reporting rests with the physician; moreover, laboratories (PHL 2102), school nurses (10NYCRR 2.12), day care center directors, nursing homes/hospitals (10NYCRR 405.3d) and state institutions (10NYCRR 2.10a) or other locations providing health services (10NYCRR 2.12) are also required to report the diseases listed below.

|                                                                 |                                                        |                                                                    |                                                                                                                |                                                              |
|-----------------------------------------------------------------|--------------------------------------------------------|--------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------|
| Anaplasmosis                                                    | Cyclosporiasis                                         | Hospital associated infections (as defined in section 2.2 10NYCRR) | Poliovirus                                                                                                     | Streptococcal infection (invasive disease) <sup>5</sup>      |
| Amebiasis                                                       | Diphtheria                                             | Influenza, laboratory-confirmed                                    | Psittacosis                                                                                                    | Group A beta-hemolytic strep                                 |
| Animal bites for which rabies prophylaxis is given <sup>1</sup> | E.coli O157:H7 infection <sup>4</sup>                  | Legionellosis                                                      | Q Fever <sup>2</sup>                                                                                           | Group B strep                                                |
| Anthrax <sup>2</sup>                                            | Ehrlichiosis                                           | Listeriosis                                                        | Rabies <sup>1</sup>                                                                                            | Streptococcus pneumoniae                                     |
| Arboviral infection <sup>3</sup>                                | Encephalitis                                           | Lyme disease                                                       | Respiratory syncytial virus (RSV) laboratory-confirmed                                                         | Syphilis, specify stage <sup>7</sup>                         |
| Babesiosis                                                      | Foodborne Illness                                      | Malaria                                                            | Respiratory syncytial virus (RSV) pediatric fatalities                                                         | Tetanus                                                      |
| Botulism <sup>2</sup>                                           | Giardiasis                                             | Measles                                                            | Rocky Mountain spotted fever                                                                                   | Toxic shock syndrome                                         |
| Brucellosis <sup>2</sup>                                        | Glanders <sup>2</sup>                                  | Melioidosis <sup>2</sup>                                           | Rubella (including congenital rubella syndrome)                                                                | Transmissible spongiform encephalopathies <sup>8</sup> (TSE) |
| Campylobacteriosis                                              | Gonococcal infection                                   | Meningitis                                                         | Salmonellosis                                                                                                  | Trichinosis                                                  |
| Chancroid                                                       | Haemophilus influenzae <sup>5</sup> (invasive disease) | Aseptic or viral                                                   | Shigatoxin-producing E.coli <sup>4</sup> (STEC)                                                                | Tuberculosis current disease (specify site)                  |
| Chlamydia Trachomatis infection                                 | Hantavirus disease                                     | Haemophilus                                                        | Shigellosis <sup>4</sup>                                                                                       | Tularemia <sup>2</sup>                                       |
| Cholera                                                         | Hemolytic uremic syndrome                              | Meningococcal                                                      | Smallpox <sup>2</sup>                                                                                          | Typhoid                                                      |
| Coronavirus                                                     | Hepatitis A                                            | Other (specify type)                                               | Staphylococcus aureus <sup>6</sup> (due to strains showing reduced susceptibility or resistance to vancomycin) | Vaccinia disease <sup>9</sup>                                |
| COVID-19 (SARS CoV-2)                                           | Hepatitis A in a food handler                          | Meningococcemia                                                    |                                                                                                                | Varicella (not shingles)                                     |
| Severe Acute Respiratory Syndrome (SARS)                        | Hepatitis B (specify acute or chronic)                 | Mumps                                                              |                                                                                                                | Vibriosis <sup>6</sup>                                       |
| Middle East Respiratory Syndrome (MERS)                         | Hepatitis C (specify acute or chronic)                 | Pertussis                                                          |                                                                                                                | Viral hemorrhagic fever <sup>2</sup>                         |
| Cryptosporidiosis                                               | Pregnant hepatitis B carrier                           | Plague <sup>2</sup>                                                |                                                                                                                | Yersiniosis                                                  |
|                                                                 | Herpes infection, infants aged 60 days or younger      |                                                                    |                                                                                                                |                                                              |

### WHO SHOULD REPORT?

Physicians, nurses, laboratory directors, infection control practitioners, health care facilities, state institutions, schools.

### WHERE SHOULD REPORT BE MADE?

Report to local health department where patient **resides**.

**Westchester County DOH M-F 8:30 AM – 4:30 PM:**

**TB – 914-995-5374 FAX 914-995-5343**

**STI - 914-813-5115 FAX 914-813-5189**

**ALL OTHER DISEASES 914-813-5159 FAX 914-813-5182**

**914-813-5000 at all other times 24/7**

### WHEN SHOULD REPORT BE MADE?

Within 24 hours of diagnosis:

- Phone diseases with **C** symbol.
- Report all other diseases promptly to **county health department where individual resides**.
- In New York City use form PD-16.

### SPECIAL NOTES

- Diseases listed with phone **C** symbol warrant prompt action and should be reported **immediately** to local health departments by phone followed by submission of the confidential case report form (DOH-389). In NYC use case report form PD-16.
- In addition to the diseases listed above, any unusual disease (defined as a newly apparent or emerging disease or syndrome that could possibly be caused by a transmissible infectious agent or microbial toxin) is reportable.
- Outbreaks: while individual cases of some diseases (e.g., streptococcal sore throat, head lice, impetigo, scabies and pneumonia) are not reportable, a cluster or outbreak of cases of any communicable disease is a reportable event.
- **Cases of HIV infection, HIV-related illness and AIDS (Stage 3) are reportable on the Medical Provider HIV/AIDS and Partner/Contact Report Form DOH-4189. The form may be obtained by contacting:**  
 Division of Epidemiology, Evaluation and Partner Services  
 P.O. Box 2073, ESP Station  
 Albany, NY 12220-2073  
 (518) 474-4284  
 In NYC: New York City Department of Health and Mental Hygiene  
 For HIV/AIDS reporting, call: (212) 442-3388

1. Local health department must be notified prior to initiating rabies prophylaxis.
2. Diseases that are possible indicators of bioterrorism.
3. Including, but not limited to, infections caused by eastern equine encephalitis virus, western equine encephalitis virus, West Nile virus, St. Louis encephalitis virus, La Crosse virus, Powassan virus, Jamestown Canyon virus, dengue and yellow fever.
4. Positive shigatoxin test results should be reported as presumptive evidence of disease.
5. Only report cases with positive cultures from blood, CSF, joint, peritoneal or pleural fluid. Do not report cases with positive cultures from skin, saliva, sputum or throat.
6. Proposed addition to list.
7. Any non-treponemal test  $\geq 1:16$  or any positive prenatal or delivery test regardless of titer or any primary or secondary stage disease, should be reported by phone; all others may be reported by mail.
8. Including Creutzfeldt-Jakob disease. Cases should be reported directly to the New York State Department of Health Alzheimer's Disease and Other Dementias Registry at (518) 473-7817 upon suspicion of disease. In NYC, cases should also be reported to the NYCDOHMH.
9. Persons with vaccinia infection due to contact transmission and persons with the following complications from vaccination; eczema vaccinatum, erythema multiforme major or Stevens-Johnson syndrome, fetal vaccinia, generalized vaccinia, inadvertent inoculation, ocular vaccinia, post-vaccinal encephalitis or encephalomyelitis, progressive vaccinia, pyogenic infection of the infection site, and any other serious adverse events.

### ADDITIONAL INFORMATION

For more information on disease reporting, call your local health department or the New York State Department of Health Bureau of Communicable Disease Control at (518) 473-4439 or (866) 881-2809 after hours. In New York City, I (866) NYC-DOHI.

**PLEASE POST THIS CONSPICUOUSLY**